# FOLFOXIRI

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## FOLFOXIRI — chemotherapy cycle

**Active medicines:** Irinotecan + Oxaliplatin + Folinic acid + Fluorouracil

**Emetogenic risk:** High

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 before chemotherapy<br>Check interactions; national dexamethasone doses below already account for this combination. |
| Aprepitant | 80 mg orally once daily | Days 2 and 3<br>Do not duplicate another NK1 antagonist. |
| Ondansetron | 16 mg orally once | Day 1 before chemotherapy<br>Review QT risk, interactions, constipation and hepatic function. |
| Dexamethasone | 12 mg orally once | Day 1 before chemotherapy<br>Reconcile all steroids; do not duplicate. |
| Dexamethasone | 8 mg orally once daily | Days 2, 3 and 4<br>Re-anchor to the actual treatment day if delayed. |
| Olanzapine | 5 mg orally once daily | Days 1, 2, 3 and 4<br>Review sedation, falls, glucose and interactions. |
| Metoclopramide | 10 mg orally up to three times daily as required | Subsequent-day rescue; prescriber confirms duration<br>Review renal function and neurological contraindications. Concomitant olanzapine increases extrapyramidal adverse-effect risk; reconcile rescue medicines. |
| Atropine sulphate | 250 micrograms subcutaneously | Before irinotecan if prescribed<br>Avoid in glaucoma; use caution with asthma. This treats acute cholinergic effects, not delayed diarrhoea. |

### Additional phase-specific support

- This chemotherapy combination has a high risk of sickness without prevention. Follow the prescribed prevention and rescue medicines. Call if vomiting prevents drinking; do not duplicate steroids or anti-sickness doses.
- Growth-factor injections depend on your risk, blood counts and previous complications. If prescribed, confirm their timing in relation to chemotherapy and pump removal; do not assume everyone receives them.
- Atropine is normally prescribed before irinotecan for early cramps, sweating or diarrhoea. It is avoided with glaucoma; tell the team about asthma as well. At home use the separately prescribed delayed-diarrhoea instructions. This can differ from standard over-the-counter dosing. Keep the medicine and urgent-contact plan available.
- Avoid cold drinks, cold air and cold objects if they trigger symptoms. Do not use ice chips during oxaliplatin treatment. Ask for non-cold mouth-care advice. New breathing difficulty must still be assessed rather than assumed to be a cold reaction.
- Irinotecan delayed-diarrhoea rescue must be supplied as a written prescription: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool, for no longer than 48 hours at this high-dose schedule. Fever, dehydration, severe pain or persistent diarrhoea needs urgent oncology assessment. Do not carry this schedule into capecitabine treatment without a new prescription.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00555 v7 · NCCP NCIS antiemetics V8: high-risk non-AC row; multiday support reviewed by prescriber

Sources checked 2026-11-04 · SACTCheck v0.84.33 · Regimen-level drafts; prescriber and oncology-pharmacy verification required